

العمر(تاريخ الميلاد) : 29-07-1960 المهنة : ربه منزل

رئيس
المجلس الطبي المتخصص

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Name: فوزيه مصطفى عبدالحميد ابو الخير National ID: 26007292101328	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

فوزيه
مصطفى عبد الحميد ابو الخير
المنصوريه
منشأة القناطر - الجيزه

١٩٦٠/٠٧/٢٨

٢٦٠٠٧٢٩٢١٠١٣٢٨

LJ9065103

٢٠٢٥/٠٩ ٢٦٠٠٧٢٩٢١٠١٣٢٨

ربة منزل

أنثى

مسلمة

ارملة

البطاقة سارية حتى ٢٠٣٢/٠٩/١٨

١

mm/dd/yyyy



MDT FORM

Name: Fawzaya Moustfa El-Sayed Abo Elkhair	MRN: 5451
Age: 66 Y	ID:
Menopausal Status: POST	Phone: 01127666831
NRS SCORE: PS 1	Marital Status: WIDOW
BMI: 37	Family History: -VE
CO Morbidity: NAD	Reason Of MDT: POST OP FOR ADJ TTT

Clinical Data:

BX: IDC G1
ER 8/8 **PR** 8/8 **HER** 0 **KI67** 5

R

L

Sonomammography:

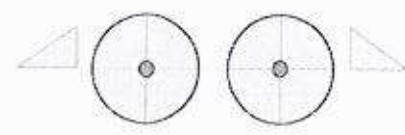
UIFOCAL BREAST CANCER IN UIQ OF LT BREAST
 ACTUAL SIZE AFTER OPERATION (1.3/0.8/0.5)

Mets Workup:

CT cap..... free (M0)

History:

LT WLE + SLNB FREE MARGINS..... LN 0/5.....
 pT1c pN0



T N M

Recommendations:

MDT Outcome & Plan

PORT
 Femara x 5 years
 Zometa / 6m

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. هبة عبد العزيز	د. محمد الشوي				

Patient Name	: الأستاذه / فوزيه مصطفى عبدالحميد	Request No.	: 13002993
Patient No.	: 26007292101328	Request Date	: 11/11/2025
Path No.	: 3473/25	Verification Date	: 18/11/2025
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 65 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data

Left breast UIQ mass.

Nature of specimen

Truc tut biopsy collected on 11/11/2025

Gross examination

Fragmented grey white tissue core ranging in size from 0.2 cm up to 1 cm, totally submitted.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against: HER2/NEU (4B5), KI67 (30-9), PR (1E2), ER (SP1).

Microscopic examination

- * Histological type: Invasive duct carcinoma .
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (2+2+1) Grade 1
- * DCIS: Absent
- * Lobular intra epithelial neoplasia : Absent
- * Angioinvasion: Absent .
- * Microcalcification : Positive
- * Perineural invasion: Absent
- IPT (Positive control showed appropriate reactivity) :
- * ER: Strong nuclear reaction in 98 % tumor cells (Allred score 8/8)
- * PR: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)
- * HER2: Faint incomplete membrane reaction in <10 % of tumor cells (ultra-low)
- * Ki 67: 5 %

Verified by :

Dr.Heba Hasan

Printing Date : 24/11/2025 13:56:55

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / فوزيه مصطفى عبدالحمد	Request No. : 13002993
Patient No. : 26007292101328	Request Date : 11/11/2025
Path No. : 3473/25	Verification Date : 18/11/2025
Sex : Female	Financial Category : جوستاف روسي - مبادره
Age : 65 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

Pathology Report

Diagnosis

LEFT BREAST MASS, TRU CUT BIOPSY :

-Invasive duct carcinoma Grade 1 (tubular differentiation 2, pleomorphism 2, mitosis 1)
(according to Nottingham grading system of breast cancer).

IPT:

* ER: Positive (score 8/8).

* PR: Positive (score 8/8).

* HER2:Ultra-low

* Ki67: 5 %

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr.Walaa Ibrahim Ghoneim

Dr.Heba El-Shafeey

Verified by :

Dr.Heba Hasan

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Patient ID: 89957	Patient Name English: Fawziah Mostafa Abd Alhamid
Age: ٦٥ سنة, ٢ شهر	2025/11/09 09:58:59 AM :Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 65 years old female patient with left breast pain.

Detailed Findings

Left breast:

- Heterogeneously dense breast parenchyma (ACR density c).
- The left breast shows posterior third irregular spiculated dense mass lesion with internal calcific focus, located at UIQ and about 11.5 cm from the nipple.
- Benign vascular calcification.
- Ultrasound revealed irregular, speculated hypoechoic mass measures about 2.2X1.9 cm at its maximum diameters and located at 11 O'clock position about 1.2 cm from underlying skin.
- Normal nipple/areolar complex.
- Two enlarged axillary lymph nodes at level I, the largest one is globular with non-uniform cortical outline and 0.5 cm focal cortical thickening, likely pathological. Another smaller yet globular with cortical thickening LN is noted.
- No pathological supra clavicular LNs.

Right breast:

- Heterogeneously dense breast parenchyma (ACR density c).
- No speculated masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- Benign vascular calcification.
- Ultrasound demonstrated prominent glandular elements of the breast tissue with mottled appearance.
- No cystic or solid lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- Left breast (BIRADS 5): left UIQ malignant looking mass lesion with ipsilateral pathological axillary node as described. For histopathological confirmation.
- Right breast (BIRADS 2): benign vascular calcifications.

*Much obliged,
Dr. Huda Alabd
Dr. Ghada Khaled*

Name : Fawzeya Moustafa Abdel Hameed

Date : 09-12-2025 10:27

Abol Khair

Branch : Portable

Medical No. : GRI-133297

Ref.Physician Prof :

Ins.Company :

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Degraded image quality due to poor breath hoding.

I- CHEST:

- The left breast showed focal parenchymal irregularity with bilateral prominent axillary lymph nodes for sono-mammography correlation.
- No pulmonary nodules, masses or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- The heart is mildly enlarged in size

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- Pelvic calcific pleboli.
- No ascites.
- **The examined skeleton** showed dorsolumbar spondylosis with no worrisome lytic or sclerotic lesions.

Name : Fawzeya Moustafa Abdel Hameed

Date : 09-12-2025 10:27

Abol Khair

Branch : Portable

Medical No. : GRI-133297

Ref.Physician Prof :

Ins.Company :

Opinion:

- Left breast focal parenchymal irregularity with bilateral prominent axillary lymph nodes for further sono-mammography.
- Mild cardiomegaly.
- Rest of examination is unremarkable

Dr.Norhan Ramzy,MD

Dr.NOURHAN RAMZY

Dr.NOURHAN RAMZY